ICU Admission Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0014	**Age / Sex**	28 / M
Setting	Neuro-ICU	**Primary dx**	Severe TBI, diffuse axonal injury

Chief complaint / reason for admission

Severe head injury in a high-speed car crash with persistent coma and no mass lesion on CT.

History of present illness

Restrained driver in a head-on collision at approximately 90 km/h. Unconscious at the scene, extricated after 25 minutes. Intubated by EMS because of low GCS and airway compromise.

Past history / medications

Previously healthy, no medications.

Examination

Ventilated, sedated for transport. Pupils 3 mm, reactive and symmetrical. Facial lacerations and a left rib fracture. Tone increased in all limbs. No sustained clonus.

Glasgow Coma Scale

At the scene: eyes open to pain only, moaning incomprehensibly, abnormal flexion (decorticate) to pain. On day 5 after sedation was stopped: eyes open to voice, grunts without words, withdraws from painful stimulus.

GCS at scene (EMS)	Score	Response
Eye opening (E)	2	To pain
Verbal (V)	2	2 - Incomprehensible sounds
Motor (M)	3	Abnormal flexion (decorticate)
TOTAL GCS		**7**

GCS on ICU day 5 (sedation off)	Score	Response
Eye opening (E)	3	To voice
Verbal (V)	2	2 - Incomprehensible sounds
Motor (M)	4	Withdraws from pain
TOTAL GCS		**9**

Investigations

CT head: small punctate hemorrhages at the gray-white junction and corpus callosum, mild diffuse swelling, no extra-axial collection. MRI SWI at day 4: numerous microbleeds in the splenium, bilateral frontal and brainstem dorsolateral regions (Adams grade 3). ICP 14 mmHg. EEG diffuse slowing.

Assessment

Severe diffuse axonal injury with prolonged disorder of consciousness, no surgical lesion. Initial GCS 7, improving slowly.

Plan / disposition

ICP monitoring, normocapnia, sodium 140-145, early tracheostomy planning, enteral nutrition, DVT prophylaxis. Early neurorehabilitation. Family counseled on prolonged recovery and uncertain outcome.